Tigelat 50mg (Tigecycline Lyophilised Powder for Intravenous Infusion 50mg/vial)

Description & Composition:

Tigelat 50mg (Tigecycline Lyophilised Powder for Intravenous Infusion 50mg/vial, 5ml Lyo vial) is an orange lyophilised cake or powder filled in 5ml USP Type 1 clear tubular glass lyo vial administered by IV infusion.

Description after dilution:

Tigelat 50mg is diluted with 5.3ml of 0.9% sodium chloride injection, 5% dextrose injection or Lactated Ringer's injection to achieve a concentration of 10mg/ml of Tigecycline.

Dilution in 0.9% sodium chloride injection - A clear, yellow to orange colour solution

Dilution in 5% dextrose injection - A clear, yellow to orange colour solution

Dilution in Lactated Ringer's injection - A clear, yellow to orange colour solution

CLINICAL PARTICULARS

INDICATIONS

Tigecycline is indicated for the treatment of the following infections caused by susceptible strains of the designated microorganisms in the conditions listed below for patients 18 years of age and older.

Complicated skin and skin structure infections caused by Escherichia coli, Enterococcus faecalis (vancomycin-susceptible isolates only), Staphylococcus aureus (methicillin-susceptible and – resistant isolates), Streptococcus agalactiae, Streptococcus anginosus grp. (includes S. anginosus, S. intermedius,and S. constellatus), Streptococcus pyogenes, Enterobacter cloacae, Klebsiella pneumoniae, and Bacteroides fragilis.

Complicated intra-abdominal infections caused by Citrobacter freundii, Enterobacter cloacae, E. coli, Klebsiella oxytoca, Klebsiella pneumoniae, Enterococcus faecalis (vancomycin-susceptible isolates), Staphylococcus aureus (methicillin-susceptible and -resistant isolates only), Streptococcus anginosus grp. (includes S. anginosus, S. intermedius, and S. constellatus), Bacteroides fragilis, Bacteroides thetaiotaomicron, Bacteroides uniformis, Bacteroides vulgatus, Clostridium perfringens, and Peptostreptococcus micros.

Appropriate specimens for bacteriological examination should be obtained in order to isolate and identify the causative organisms and to determine their susceptibility to tigecycline. Tigecycline may be initiated as empiric monotherapy before results of these tests are known. To reduce the development of drug-resistant bacteria and maintain the effectiveness of tigecycline and other antibacterial drugs, tigecycline should be used only to treat infections that are proven or strongly suspected to be caused by susceptible bacteria. When culture and susceptibility information are available, they should be considered in selecting or modifying antibacterial therapy. In the absence of such data, local epidemiology and susceptibility patterns may contribute to the empiric selection of therapy.

POSOLOGY AND METHOD OF ADMINISTRATION

Dosage

The recommended dosage regimen for tigecycline is an initial dose of 100 mg, followed by 50 mg every 12 hours. Intravenous (IV) infusions of tigecycline should be administered over approximately 30 to 60 minutes every 12 hours.

The recommended duration of treatment with tigecycline for complicated skin and skin structure infections (cSSSI) or for complicated intra-abdominal infections (cIAI) is 5 to 14 days. The duration of therapy should be guided by the severity and site of the infection and the patient's clinical and bacteriological progress.

Use in patients with renal impairment

No dosage adjustment of tigecycline is necessary in patients with renal impairment or in patients undergoing hemodialysis.

Use in patients with hepatic impairment

No dosage adjustment is necessary in patients with mild to moderate hepatic impairment (Child-Pugh A and Child Pugh B). Based on the pharmacokinetic profile of tigecycline in patients with severe hepatic impairment (Child Pugh C), the dose of tigecycline should be altered to 100 mg followed by 25 mg every 12 hours. Patients with severe hepatic impairment (Child Pugh C) should be treated with caution and monitored for treatment response.

Use in children

Clinical trials to establish the safety and effectiveness of tigecycline in patients under 18 years of agehave not been conducted. Therefore, use in patients under 18 years of age is not recommended.
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Use in elderly

No dosage adjustment is necessary in elderly patients.

Race and gender

No dosage adjustment is necessary based on race or gender.

Mode of administration

Intravenous infusion

Instructions for use

Each vial of Tigelat 50mg should be reconstituted with 5.3 mL of 0.9% Sodium Chloride Injection, USP or 5% Dextrose Injection, USP or Lactated Ringers Solution to achieve a concentration of 10 mg/mL of tigecycline. (Note: Each vial contains a 6% overage, thus, 5mL of reconstituted solution is equivalent to 50mg of the drug.) The vial should be gently swirled until the drug dissolves. Withdraw 5 mL of the reconstituted solution from the vial and add to a 100 mL intravenous bag for infusion (for a 100 mg dose, reconstitute two vials; for a 50 mg dose, reconstitute one vial). The maximum concentration in the intravenous bag should be 1 mg/ mL. The reconstituted solution should be yellow to orange in colour; if not, the solution should be discarded. Parenteral drug products should be inspected visually for particulate matter and discoloration (e.g., green or black) prior to administration.

The powder should be reconstituted immediately after opening of the vial. Once reconstituted, Tigelat 50 mixed with 0.9% sodium chloride injection, 5% dextrose injection and Lactated Ringer's solution may be stored up to 6 hours at 25C in the vial.

After reconstitution and dilution, chemical and physical stability has been demonstrated for 18 hours at 25C/60%RH and 48 hours at 2-8C in polyethylene bags. From a microbiological point of view, the solution should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 48 hours at 2-8C and 18 hours at 25C/60%RH. unless reconstitution/dilution has taken place in a controlled and validated aseptic condition.

Tigelat 50 may be administered intravenously through a dedicated line or through a Y- site. If the same intravenous line is used for sequential infusion of several drugs, the line should be flushed before and after infusion of Tigelat 50 with 0.9% Sodium Chloride Injection, or 5% Dextrose Injection or Lactated Ringers Solution. Injection should be made with an infusion solution compatible with tigecycline and with any other drug(s) administered via this common line.

Compatibilities
Compatible intravenous solutions include sodium chloride 9 mg/ml (0.9 %) solution for injection, dextrose 50 mg/ml (5 %) solution for injection, and Lactated Ringer’s solution for injection. When administered through a Y-site, Tigelat 50mg is compatible with the following drugs or diluents when used with either 0.9% Sodium Chloride Injection, USP or 5% Dextrose Injection, USP: amikacin, dobutamine, dopamine HCl, gentamicin, haloperidol, Lactated Ringer’s, lidocaine HCl, metoclopramide, morphine, norepinephrine, piperacillin/ tazobactam (EDTA formulation), potassium chloride, propofol, ranitidine HCl, theophylline, and tobramycin.

Incompatibilities
The following drugs should not be administered simultaneously through the same Y-site as Tigelat 50mg: amphotericin B, amphotericin B lipid complex, diazepam, esomeprazole and omeprazole."

CONTRAINDICATIONS

- Hypersensitivity to the active substance or to any of the excipients.
- Patients hypersensitive to tetracycline class antibiotics may be hypersensitive to tigecycline.

SPECIAL WARNINGS AND SPECIAL PRECAUTIONS FOR USE

In clinical studies in complicated skin and soft tissue infections (cSSTI), complicated intra-abdominal infections(cIAI), diabetic foot infections, nosocomial pneumonia and studies in resistant pathogens, a numerically higher mortality rate among Tigecycline treated patients has been observed as compared to the comparator treatment. The causes of these findings remain unknown, but poorer efficacy and safety than the study comparators cannot be ruled out.

Superinfection
In clinical trials in cIAI patients, impaired healing of the surgical wound has been associated with superinfection. A patient developing impaired healing should be monitored for the detection of superinfection.

Patients who develop super-infections, in particular nosocomial pneumonia, appear to be associated with poorer outcomes. Patients should be closely monitored for the development of super-infection. If a focus of infection other than cSSTI or cIAI is identified after initiation of Tigecycline therapy consideration should be given to instituting alternative antibacterial therapy that has been demonstrated to be efficacious in the treatment of the specific type of infection(s) present.

Anaphylaxis

Anaphylaxis/anaphylactoid reactions, potentially life-threatening, have been reported with tigecycline.

Hepatic failure

Cases of liver injury with a predominantly cholestatic pattern have been reported in patients receiving tigecycline treatment, including some cases of hepatic failure with a fatal outcome. Although hepatic failure may occur in patients treated with tigecycline due to the underlying conditions or concomitant medicinal products, a possible contribution of tigecycline should be considered.

Tetracycline class antibiotics.

Glycylcycline class antibiotics are structurally similar to tetracycline class antibiotics. Tigecycline may have adverse reactions similar to tetracycline class antibiotics. Such reactions may include photosensitivity, pseudotumor cerebri, pancreatitis, and anti-anabolic action which has led to increased BUN, azotaemia, acidosis, and hyperphosphataemia.

Pancreatitis

Acute pancreatitis, which can be serious, has occurred (frequency: uncommon) in association with tigecycline treatment. The diagnosis of acute pancreatitis should be considered in patients taking tigecycline who develop clinical symptoms, signs, or laboratory abnormalities suggestive of acute pancreatitis. Most of the reported cases developed after at least one week of treatment. Cases have been reported in patients without known risk factors for pancreatitis. Patients usually improve after tigecycline discontinuation. Consideration should be given to the cessation of the treatment with tigecycline in cases suspected of having developed pancreatitis.

Underlying disease

Experience in the use of tigecycline for treatment of infections in patients with severe underlying diseases is limited.

Consideration should be given to the use of combination antibacterial therapy whenever tigecycline is to be administered to severely ill patients with complicated intra -abdominal infections (cIAI) secondary to clinically apparent intestinal perforation or patients with incipient sepsis or septic shock.

The effect of cholestasis in the pharmacokinetics of tigecycline has not been properly established. Biliary excretion accounts for approximately 50 % of the total tigecycline excretion. Therefore, patients presenting with cholestasis should be closely monitored.

Prothrombin time or other suitable anticoagulation test should be used to monitor patients if tigecycline is administered with anticoagulants.

Pseudomembranous colitis has been reported with nearly all antibacterial agents and may range in severity from mild to life threatening. Therefore, it is important to consider this diagnosis in patientswho present with diarrhoea during or subsequent to the administration of any antibacterial.

The use of tigecycline may result in overgrowth of non-susceptible organisms, including fungi. Patients should be carefully monitored during therapy.

Results of studies in rats with tigecycline have shown bone discolouration. Tigecycline may be associated with permanent tooth discolouration in humans if used during tooth development.

Paediatric population

Clinical experience in the use of tigecycline for the treatment of infections in paediatric patients aged 8 years and older is very limited. Consequently, use in children should be restricted to those clinical situations where no alternative antibacterial therapy is available.

Nausea and vomiting are very common adverse reactions in children and adolescents. Attention should be paid to possible dehydration. Tigecycline should be preferably administered over a 60-minute length of infusion in paediatric patients.

Abdominal pain is commonly reported in children as it is in adults. Abdominal pain may be indicative of pancreatitis. If pancreatitis develops, treatment with tigecycline should be discontinued.

Liver function tests, coagulation parameters, haematology parameters, amylase and lipase should be monitored prior to treatment initiation with tigecycline and regularly while on treatment.

Tigelat 50mg injection should not be used in children under 8 years of age due to the lack of safety and efficacy data in this age group and because tigecycline may be associated with permanent teeth discoloration.

DRUG INTERACTION

Interaction studies have only been performed in adults.

Concomitant administration of tigecycline and warfarin (25 mg single-dose) to healthy subjects resulted in a decrease in clearance of R-warfarin and S-warfarin by 40 % and 23 %, and an increase in AUC by 68 % and 29 %, respectively. The mechanism of this interaction is still not elucidated. Available data does not suggest that this interaction may result in significant INR changes. However, since tigecycline may prolong both prothrombin time (PT) and activated partial thromboplastin time (aPTT), the relevant coagulation tests should be closely monitored when tigecycline is co-administered with anticoagulants. Warfarin did not affect the pharmacokinetic profile of tigecycline.

Tigecycline is not extensively metabolised. Therefore, clearance of tigecycline is not expected to be affected by active substances that inhibit or induce the activity of the CYP450 isoforms. In vitro, tigecycline is neither a competitive inhibitor nor an irreversible inhibitor of CYP450 enzymes.

Tigecycline in recommended dosage did not affect the rate or extent of absorption, or clearance of digoxin (0.5 mg followed by 0.25 mg daily) when administered in healthy adults. Digoxin did not affect the pharmacokinetic profile of tigecycline. Therefore, no dosage adjustment is necessary when tigecycline is administered with digoxin.

In in vitro studies, no antagonism has been observed between tigecycline and other commonly used antibiotic classes.

Concurrent use of antibiotics with oral contraceptives may render oral contraceptives less effective.

USE DURING PREGNANCY & LACTATION

Pregnancy:
Tigecycline may cause fetal harm when administered to a pregnant woman. Results of animal studies indicate that tigecycline crosses the placenta and is found in fetal tissues. Decreased fetal weights in rats and rabbits (with associated delays in ossification) and fetal loss in rabbits have been observed with tigecycline.

Tigecycline was not teratogenic in the rat or rabbit. In preclinical safety studies, 14C-labeled tigecycline crossed the placenta and was found in fetal tissues, including fetal bony structures. The administration of tigecycline was associated with slight reductions in fetal weights and an increased incidence of minor skeletal anomalies (delays in bone ossification) at exposures of 4.7 times and 1.1 times the human daily dose based on AUC in rats and rabbits, respectively. An increased incidence of fetal loss was observed at exposures of 1.1 times the human daily dose based on AUC in rabbits, at dosages producing minimal maternal toxicity.

There are no adequate and well-controlled studies of tigecycline in pregnant women. Tigecycline should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Tigecycline has not been studied for use during labor and delivery.

Breastfeeding:

Results from animal studies using 14C-labeled tigecycline indicate that tigecycline is excreted readily via the milk of lactating rats. Consistent with the limited oral bioavailability of tigecycline, there is little or no systemic exposure to tigecycline in nursing pups as a result of exposure via maternal milk.

It is not known whether this drug is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when tigecycline is administered to a nursing woman.

EFFECTS ON ABILITY TO DRIVE AND TO USE MACHINES

Tigecycline can cause dizziness, which may impair the ability to drive and/or operate machinery.

SIDE EFFECTS / ADVERSE REACTIONS

Frequency categories are expressed as: Very common; Common; Uncommon; Rare; Very rare; Not known (cannot be estimated from the available data).

List of adverse reactions
<u>Infections and infestations:</u>
Common: sepsis/septic shock, pneumonia, abscess, infections

<u>Blood and the lymphatic system disorders:</u>
Common: Prolonged activated partial thromboplastin time (aPTT), Prolonged prothrombin time (PT)
Uncommon: Thrombocytopenia, Increased International Normalised Ratio (INR)
Not Known : Hypofibrinogenaemia

<u>Immune system disorders:</u>
Not known: Anaphylaxis/anaphylactoid reactions.

<u>Metabolism and nutrition disorders:</u>
Common: Hypoglycaemia,
hypoproteinaemia

Nervous system disorders:
Common: Dizziness

Vascular disorders:
Common: Phlebitis
Uncommon:
Thrombophlebitis

Gastrointestinal disorders:
Very common: Nausea, vomiting,
diarrhoea
Common: Abdominal pain, dyspepsia,
anorexia
Uncommon: Acutepancreatitis.

Hepatobiliary disorders:
Common: Elevated aspartate aminotransferase (AST) in serum, and elevated alanine aminotransferase (ALT) inserum, hyperbilirubinaemia
Uncommon: Jaundice, liver injury, mostly
cholestatic
Not known: Hepatic failure.

Skin and subcutaneous tissue disorders:
Common: Pruritus, rash
Not known: Severe skin reactions, including Stevens-Johnson Syndrome

General disorders and administration site conditions:
Common: impaired healing, injection site reaction, headache
Uncommon: injection site inflammation, injection site pain, injection site oedema, injection site phlebitis

Investigations:
Common: Elevated amylase in serum, increased blood urea nitrogen (BUN)

Antibiotic Class Effects:
Pseudomembranous colitis which may range in severity from mild to life
threatening
Overgrowth of non-susceptible organisms, including fungi.

Tetracycline Class Effects:
Glycylcycline class antibiotics are structurally similar to tetracycline class antibiotics. Tetracycline class adverse reactions may include photosensitivity, pseudotumour cerebri, pancreatitis, and anti-anabolic action which has led to increased BUN, azotaemia, acidosis, and hyperphosphataemia
Tigecycline may be associated with permanent tooth discolouration if used during tooth development.

Paediatric population
Very limited safety data were available. No new or unexpected safety concerns were observed compared to that of adult

OVERDOSE:

No specific information is available on the treatment of overdosage. Intravenous administration of tigecycline at a single dose of 300 mg over 60 minutes in healthy volunteers resulted in an increased incidence of nausea and vomiting.

Tigecycline is not removed in significant quantities by haemodialysis.

PHARMACOLOGY:

Pharmacodynamic properties

Pharmacotherapeutic group: Antibacterial for systemic use, Tetracyclines, ATC

code: J01AA12.Mechanism of action

Tigecycline, a glycylcycline antibiotic, inhibits protein translation in bacteria by binding to the 30S ribosomal subunit and blocking entry of amino-acyl tRNA molecules into the A site of the ribosome. This prevents incorporation of amino acid residues into elongating peptide chains.

Tigecycline carries a glycylamido moiety attached to the 9-position of minocycline. The substitution pattern is not present in any naturally occurring or semisynthetic tetracycline and imparts certain microbiologic properties to tigecycline.

In addition, tigecycline is able to overcome the two major tetracycline resistance mechanisms, ribosomal protection and efflux. Accordingly, tigecycline has activity against a broad spectrum of bacterial pathogens. There has been no cross resistance observed between tigecycline and other antibiotics. No antagonism between tigecycline and other commonly used antibiotics. In general, tigecycline is considered bacteriostatic; however, Tigelat 50mg has demonstrated bactericidal activity against isolates of S. pneumoniae and L. pneumophila.

Mechanism(s) of Resistance
to date there has been no cross-resistance observed between tigecycline and other antibacterial. Tigecycline is not affected by the two major tetracycline-resistance mechanisms, ribosomal protection and efflux. Additionally, tigecycline is not affected by resistance mechanisms such as beta-lactamases (including extended spectrum beta- lactamases), target-site modifications, macrolide efflux pumps or enzyme target changes (e.g. gyrase/topoisomerases). Tigecycline resistance in some bacteria (e.g. Acinetobacter calcoaceticus-Acinetobacter baumannii complex) is associated with multi-drug resistant (MDR) efflux pumps.

Interaction with Other Antimicrobials
In vitro studies have not demonstrated antagonism between tigecycline and other commonly used antibacterial.

Tigecycline has been shown to be active against most of the following bacteria, both in vitro and in clinical infections.

Aerobic and Facultative Gram- Positive Microorganism
Enterococcus faecalis (vancomycin-susceptible isolates)
Staphylococcus aureus (methicillin-susceptible and -resistant isolates) Streptococcus agalactiae
Streptococcus anginosus grp. (Includes S. anginosus, S. intermedius, and S. constellatus)
Streptococcus pyogenes

Aerobic and Facultative Gram Negative
Citrobacter freundii
Enterobacter cloacae
Escherichia coli
Klebsiella oxytoca
Klebsiella pneumoniae

Anaerobic Microorganism
Bacteroides fragilis
Bacteroides thetaiotaomicron
Bacteroides uniformis
Bacteroides vulgatus
Clostridium perfringens
Peptostreptococcus micros

The following bacteria exhibit minimum inhibitory concentrations (MICs) that are at concentrations that are achievable using the prescribed dosing regimens. However, the clinical significance of this is unknown because the safety and effectiveness of tigecycline in treating clinical infections due to these bacteria have not been established in adequate and well-controlled clinical trials.

Aerobic and Facultative Gram – Positive MicroorganismEnterococcus avium
Enterococcus casseliflavus
Enterococcus faecalis (vancomycin-resistant isolates)
Enterococcus faecium (vancomycin-susceptible and -resistant isolates) Enterococcus gallinarumListeria monocytogenes
Staphylococcus epidermidis (methicillin-susceptible and -resistant isolates)
Staphylococcus haemolyticus

Aerobic and Facultative Gram – Negative
Microorganism
Acinetobacter baumannii
Aeromonas hydrophila
Citrobacter koseri
Enterobacter aerogenes
Pasteurella multocida
Serratia marcescens
Stenotrophomonas maltophilia

Anaerobic Microorganism
Bacteroides distasonis
Bacteroides ovatus
Peptostreptococcus sp.
Porphyromonas sp.
Prevotella sp.

Other bacteria
Mycobacterium abscessus
Mycobacterium chelonae
Mycobacterium fortuitum

The development of tigecycline resistance in Acinetobacter infections seen during the course of standard treatment. Such resistance appears to be attributable to an MDR efflux pump mechanism. While monitoring for relapse of infection is important for all infected patients, more frequent monitoring in this case is suggested. If relapse is suspected, blood and other specimens should be obtained and cultured for the presence of bacteria. All bacterial isolates should be identified and tested for susceptibility to tigecycline and other appropriate antimicrobials.

Pharmacokinetic properties

Absorption
Tigecycline is administered intravenously and therefore has 100 % bioavailability.

Distribution
The plasma protein binding of tigecycline ranges from approximately 71 % to 89 % at concentrations observed in clinical studies (0.1 to 1.0 μg/ml) and the steady-state volume of distribution of tigecycline averaged 500 to 700 L (7 to 9 L/kg), indicating that tigecycline is extensively distributed beyond the plasma volume and concentrates into tissues.

Biotransformation
On average, it is estimated that less than 20 % of tigecycline is metabolized before excretion.

Elimination
The total clearance of tigecycline is 24 L/h after intravenous infusion. Renal clearance is approximately 13 % of total clearance. Tigecycline shows a poly exponential elimination from serum with a mean terminal elimination half-life after multiple doses of 42 hours although high inter individual variability exists.

Special populations

Patient with Hepatic Insufficiency
The single-dose pharmacokinetic disposition of tigecycline was not altered in patients with mild hepatic impairment. However, systemic clearance of tigecycline was reduced by 25 % and 55 % and the half-life of tigecycline was prolonged by 23 % and 43 % in patients with moderate or severe hepatic impairment (Child Pugh B and C), respectively.

Patient with Renal Insufficiency
The single dose pharmacokinetic disposition of tigecycline was not altered in patients with renal insufficiency (creatinine clearance <30 ml/min, n=6). In severe renal impairment, AUC was 30 % higher than in subjects with normal renal function.

Patient with Elderly Patients
No overall differences in pharmacokinetics were observed between healthy elderly subjects and younger subjects.

Patient with Paediatric Population
The safety and efficacy of tigecycline in the paediatric population 8 to <18 years of age have not been established.

Gender
There were no clinically relevant differences in the clearance of tigecycline between men and women. AUC was estimated to be 20 % higher in females than in males.

Race
There were no differences in the clearance of tigecycline based on race.

Weight
Clearance, weight-normalised clearance, and AUC were not appreciably different among patients with different body weights, including those weighing ≥125 kg. AUC was 24 % lower in patients weighing ≥125 kg. No data is available for patients weighing 140 kg and more.

Incompatibilities

The following drugs should not be administered simultaneously through the same Y-site as Tigelat 50: amphotericin B, amphotericin B lipid complex, diazepam, esomeprazole and omeprazole.

Shelf Life

Unopened vial: 2 years

The powder should be reconstituted immediately after opening of the vial. Once reconstituted, Tigelat 50 mixed with 0.9% sodium chloride injection, 5% dextrose injection and Lactated Ringer's solution may be stored up to 6 hours at 25C in the vial.

Solution for infusion:
After reconstitution and dilution, chemical and physical stability has been demonstrated for 18 hours at 25C/60%RH and 48 hours at 2-8C in polyethylene bags. From a microbiological point of view, the solution should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 48 hours at 2-8C and 18 hours at 25C/60%RH. unless reconstitution/dilution has taken place in a controlled and validated aseptic condition.

Special Precautions for storage

Storage condition for unopened vial: Store below 30C.

The powder should be reconstituted immediately upon opening of the vial. Once reconstituted with 0.9% sodium chloride injection, 5% dextrose injection or Lactated Ringer's solution, it may be stored up to 6 hours at 25C in the vial.

Solution for infusion:
After reconstitution and dilution, chemical and physical stability has been demonstrated for 18 hours at 25C/60%RH and 48 hours at 2-8C in polyethylene bags. From a microbiological point of view, the solution should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 48 hours at 2-8C and 18 hours at 25C/60%RH. unless reconstitution/dilution has taken place in a controlled and validated aseptic condition.

Nature and Contents of Container
5ml USP type 1 clear tubular glass lyo vial, stoppered with 13mm sterilized grey bromobutyl lyo rubber stoppers and sealed with 13mm aluminium flip off seal. Each vial is packed in monocarton along with leaflet and without diluents.

Manufactured by:
Gland Pharma Limited,
Survey No 143 to 148, 150 &151
Near Gandimaisamma Cross Roads,
D.P. Pally, Dundigal post
Dundigal-Gandimaisamma Mandal
Medchal - Malkajgiri District,
Hyderabad- 500 043, Telangana, INDIA.

Product Registration Holder:
Zulat Pharmacy Sdn Bhd
23 & 23A, Jalan Bandar 3,
Taman Melawati,
53100 Kuala Lumpur Malaysia.

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